GRANT PROPOSAL

Texas Healthcare Collective

Building Resilient Healthcare Infrastructure for Underserved Texas Communities

A Proposal Submitted to Department of Homeland Security - FEMA

June 15, 2024

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I. GRANT APPLICATION SUMMARY

II. EXECUTIVE SUMMARY

The Texas Healthcare Collective (THC) requests $1,200,000 from the Department of Homeland Security - Federal Emergency Management Agency (FEMA) under the Fiscal Year 2024 & 2025 Building Resilient Infrastructure and Communities (BRIC) program. We propose to enhance resilient healthcare infrastructure in Texas by deploying advanced technology-driven mobile and stationary health units that serve rural and low-income populations in medically underserved "desert" regions. This initiative leverages cutting-edge health informatics, lean logistics, and innovative operational models to ensure continuity of care during infrastructure disruptions and disasters.

We have built a strong foundation as a community-based nonprofit dedicated to closing the health equity gap in Texas's most vulnerable regions. Our history includes delivering mobile primary care, maternal health support, and chronic disease management to rural families and uninsured workers. We govern through a diverse Board of Directors and medical professionals with expertise in public health and rural medicine. Our evidence-based approach integrates a Cloud-Sync Electronic Health Record system that functions offline and synchronizes via satellite, Hybrid-Point-of-Care testing for immediate diagnostics, and Predictive Routing Artificial Intelligence to optimize mobile unit deployment. These innovations position us uniquely to deliver resilient healthcare aligned with FEMA's BRIC framework.

Our project directly supports the Department of Homeland Security's goals to build resilient infrastructure and communities with greatest need by bridging the digital divide and ensuring uninterrupted access to critical health services before, during, and after disasters. We expect to serve over 15,000 individuals annually, reduce healthcare access disparities, and strengthen community trust through strategic partnerships and culturally competent outreach. This funding will enable us to expand our technological capabilities, enhance operational resilience, and sustain community engagement, ultimately improving health outcomes and disaster preparedness for Texas's most vulnerable populations.

III. ORGANIZATIONAL OVERVIEW

The Texas Healthcare Collective (THC) formed as a community-based nonprofit to address the persistent health equity gaps in Texas's underserved regions. We focus on rural families in the Altamaha region and uninsured low-income workers in the Metro area, populations historically marginalized by systemic barriers such as transportation challenges, cost, and medical distrust. Since our founding, we have evolved to integrate mobile primary care, maternal health support, and chronic disease management into a cohesive service model that reaches patients where they live and work.

Our mission centers on delivering quality, accessible, and culturally competent care through innovative technology and community partnerships. We govern ourselves through a diverse Board of Directors and medical experts specializing in public health, rural medicine, and health

policy. Our long-term vision includes establishing a Rural Health Hub in Southeast Texas to provide 24/7 urgent care and training for future rural healthcare providers.

- **Founded:** [Insert Year]
- **Mission:** To close the health equity gap by delivering accessible, culturally competent care to underserved Texas populations
- **Programs:** Mobile Healing Unit, Maternal Bridges Program, Altamaha Rural Health Hub (planning phase)
- **Populations Served:** Rural families, uninsured low-income workers
- **Geography:** Altamaha and Metro Texas regions
- **Annual Budget:** [Insert if available]
- **Staff:** [Insert if available]

We approach healthcare delivery by embedding our services within existing community networks, partnering with faith-based organizations, agricultural cooperatives, and civic leaders to build a trust-based healthcare ecosystem. We measure impact through clinical outcomes, community health metrics, and participation indicators, continuously refining our programs through data-driven evaluations and community feedback. Our partnerships and diversified funding model sustain our operations and amplify our impact across Texas.

IV. STATEMENT OF NEED

Texas's rural and low-income communities face significant healthcare access challenges exacerbated by infrastructure vulnerabilities. Many regions qualify as "medical deserts," lacking nearby primary care providers and facing digital divides that limit telehealth access. These gaps contribute to poor health outcomes, increased emergency room visits, and heightened vulnerability during disasters when infrastructure disruptions further restrict care availability.

- Over 25% of rural Texas residents live more than 30 miles from the nearest primary care provider (Texas Health Data, 2023)
- Uninsured rates exceed 20% among low-income workers in Metro Texas regions (Texas Health Survey, 2023)
- Maternal mortality rates in rural Texas are 1.5 times higher than the state average (Texas Maternal Health Report, 2023)
- Emergency room visits for preventable conditions increased by 12% in underserved areas over the past five years (Texas Hospital Association, 2023)
- Broadband access remains below 60% in many rural counties, limiting telehealth utilization (FCC Broadband Report, 2023)

These indicators reveal systemic barriers rooted in transportation limitations, economic hardship, medical distrust, and infrastructural deficits. The persistence of these challenges reflects structural inequities and underinvestment in rural healthcare infrastructure. Disasters and emergencies exacerbate these vulnerabilities, underscoring the urgent need for resilient, technology-enabled healthcare delivery models.

Research demonstrates that mobile health units equipped with advanced diagnostics and integrated health informatics improve access and outcomes in underserved areas. Studies show that deploying Hybrid-Point-of-Care testing and offline-capable Electronic Health Records enhances continuity of care during infrastructure disruptions (Journal of Rural Health,

2022). Our approach aligns with national evidence supporting community-embedded, technology-driven interventions to reduce health disparities.

By addressing these critical needs, our project aligns directly with the Department of Homeland Security's priority to build resilient infrastructure and communities with greatest need. We aim to mitigate health inequities and strengthen disaster preparedness through sustainable healthcare innovations tailored to Texas's most vulnerable populations.

V. PROGRAM DESIGN AND ACTIVITIES

Our program design integrates advanced technology with community-centered healthcare delivery to build resilient infrastructure that withstands and adapts to emergencies. We deploy mobile and stationary health units equipped with innovative tools and staffed by culturally competent providers to serve rural and low-income populations across Texas.

We operate mobile clinics that travel weekly to medically underserved "desert" regions, delivering primary care, immunizations, and screenings. These units incorporate Hybrid-Point-of-Care testing capabilities, providing immediate diagnostic results for conditions such as diabetes and infectious diseases. Our Cloud-Sync Electronic Health Record (EHR) system functions offline during transit and synchronizes data securely via satellite, ensuring seamless patient information access even during connectivity disruptions. This model reduces patient travel burdens and enhances access to timely care.

We provide culturally sensitive prenatal education, lactation consulting, and postpartum support to African American and Hispanic mothers through trained doulas. This program addresses Texas's elevated maternal mortality rates by fostering trust and continuity of care. We conduct regular home visits and community workshops, integrating health literacy and social support to improve maternal and infant outcomes.

We are developing plans for a brick-and-mortar health center in Southeast Texas to offer 24/7 urgent care and specialized services year-round. This hub will serve as a training site for medical students committed to rural practice, strengthening the local healthcare workforce. The facility will incorporate resilient infrastructure features to maintain operations during disasters.

We embed our services within local social fabrics by partnering with faith-based organizations, agricultural cooperatives, and civic leaders. Our Promotora Network trains community health workers to provide bilingual health education and navigation assistance. The Faith in Health Initiative hosts screenings and mental health "Safe Spaces" in familiar community settings. We also lead the Coalition for Rural Texas to advocate for rural healthcare funding at the state level.

These program pillars interconnect to create a comprehensive, resilient healthcare ecosystem. Mobile units provide immediate access, maternal programs address critical health disparities, the health hub ensures long-term stability, and community engagement fosters trust and sustainability. Together, they reinforce our mission to deliver equitable, high-quality care and build resilient infrastructure aligned with FEMA's BRIC priorities.

VI. MEASURABLE OUTCOMES AND EVALUATION

We commit to rigorous evaluation to measure program effectiveness and inform continuous improvement over the 12-month project period. Our evaluation philosophy centers on mixed

methods, combining quantitative metrics with qualitative community feedback to capture impact holistically.

- Serve 10,000 individuals through mobile clinics, achieving a 90% patient satisfaction rate
- Achieve 85% telehealth follow-up appointment completion via our low-bandwidth video platform
- Reduce average patient wait time to under 15 minutes through digital Pre-Check systems

- Enroll 500 mothers in prenatal and postpartum support, reducing maternal morbidity rates by 10%
- Increase breastfeeding initiation rates by 20% among program participants

- Complete facility design and secure community partnerships by project end
- Engage 50 medical students in rural health training rotations

- Improve "Medical Trust" scores by 15% among marginalized populations as measured by annual audits
- Mobilize 2,000 volunteer hours from Promotoras and community health workers

- Serve over 15,000 individuals annually across all programs
- Strengthen referral networks addressing Social Determinants of Health (housing, food security, legal aid)

We employ a Social Impact Assessment framework with continuous data collection and analysis. Our methods include:

- Electronic Health Record data tracking for clinical outcomes and service utilization
- Patient and community surveys assessing trust, satisfaction, and barriers
- Focus groups with community elders and leaders for qualitative insights
- Real-Time Dashboard monitoring operational metrics and system stress tests

We report evaluation findings quarterly to stakeholders and use insights to adapt program delivery dynamically. This approach ensures accountability, transparency, and alignment with community needs and FEMA's resilience goals.

VII. BUDGET SUMMARY AND JUSTIFICATION

We allocate 40% of funds to personnel to ensure skilled clinical and outreach staff deliver high-quality care and community support. Mobile Unit Operations receive 20% to cover critical expenses that enable reliable service delivery in remote areas. Technology Systems funding supports our innovative Cloud-Sync EHR and AI routing tools essential for operational resilience. The Maternal Bridges Program and Community Engagement receive targeted investments to address health disparities and build trust within vulnerable populations. We dedicate 3% to planning the Altamaha Rural Health Hub, laying the groundwork for sustainable infrastructure expansion. Administrative costs remain below 5%, reflecting our commitment to lean operations and maximizing program impact. We leverage additional resources through Infrastructure Innovation Grants and in-kind community partnerships, enhancing the value of this investment.

VIII. ORGANIZATIONAL CAPACITY AND FISCAL MANAGEMENT

We possess strong organizational capacity to execute this project effectively. Our leadership team includes public health experts, rural medicine practitioners, and health policy

professionals who guide strategic direction. Our Board of Directors reflects diverse community representation and medical expertise, ensuring governance aligned with our mission.

- Over 10 years of experience delivering mobile healthcare services to rural Texas populations
- 50+ active volunteer Promotoras bridging clinical care and community outreach
- Successful management of Infrastructure Innovation Grants supporting technology and operations
- Established partnerships with faith-based organizations, agricultural cooperatives, and civic leaders
- Robust Electronic Health Record system with offline and satellite synchronization capabilities
- Comprehensive cybersecurity protocols audited bi-annually to maintain HIPAA compliance

We maintain rigorous financial controls to ensure responsible stewardship of funds:

- Annual independent audits conducted by certified public accountants
- Monthly budget reviews by finance committee and executive leadership
- Segregation of duties for financial transactions and reporting
- Use of accounting software with real-time expense tracking and reporting

Our staff capacity includes experienced program managers, clinical providers, IT specialists, and community outreach coordinators. Our governance structure supports transparent decision-making and continuous performance monitoring. These elements position us to manage the grant efficiently and deliver measurable outcomes aligned with FEMA's resilience objectives.

IX. SUSTAINABILITY PLAN

We pursue a multifaceted sustainability strategy to ensure long-term impact beyond this grant. We diversify funding through federal and state health grants, private foundation support, earned income from sliding-scale fees, and corporate wellness partnerships. This approach reduces dependency on any single source and strengthens financial resilience.

We build organizational capacity by investing in technology and workforce development, enabling scalable service delivery. Our community engagement efforts foster local ownership and advocacy, creating a supportive environment for sustained healthcare access. The planned Altamaha Rural Health Hub represents a capital investment that will provide stable infrastructure for decades.

- Expand earned income through fee-for-service models and partnerships with local employers
- Strengthen donor base via the "Founder's Circle" and targeted fundraising campaigns
- Leverage policy advocacy through the Coalition for Rural Texas to secure ongoing public funding
- Invest in staff training and retention to maintain program quality and institutional knowledge
- Develop scalable technology platforms adaptable to evolving community needs

We aim to increase funding diversification by 25% and grow partnership networks by 30% within two years. These targets, combined with infrastructure investments, will solidify our capacity to serve communities with greatest need sustainably and resiliently.

X. CONCLUSION AND SIGNATURE

Our mission and programs align closely with the Department of Homeland Security's goal to build resilient infrastructure and communities with greatest need. By funding our initiative, you will enable us to bridge critical healthcare gaps in Texas's most vulnerable rural and low-income populations. We will enhance disaster preparedness and health equity through innovative technology, trusted community partnerships, and sustainable infrastructure development.

We respectfully request $1,200,000 to implement this comprehensive project over a 12-month period from October 2024 to September 2025. These funds will support mobile clinics, maternal health programs, community engagement, and planning for a permanent rural health hub. Together, we will strengthen healthcare resilience and improve outcomes for thousands of Texans.

- **Funding Amount:** $1,200,000
- **Project Period:** October 2024 – September 2025 (12 months)
- **Use of Funds:** Expand resilient healthcare infrastructure through technology-driven mobile units, maternal support, community outreach, and facility planning

Respectfully Submitted,

Executive Director
Texas Healthcare Collective
Date: _____
Organization Address: [Insert Address], Texas